



University of Al-Ameed
College of Medicine

**Department of
Internal Medicine**



General Physical Examination
Professor Hassan Nassrullah

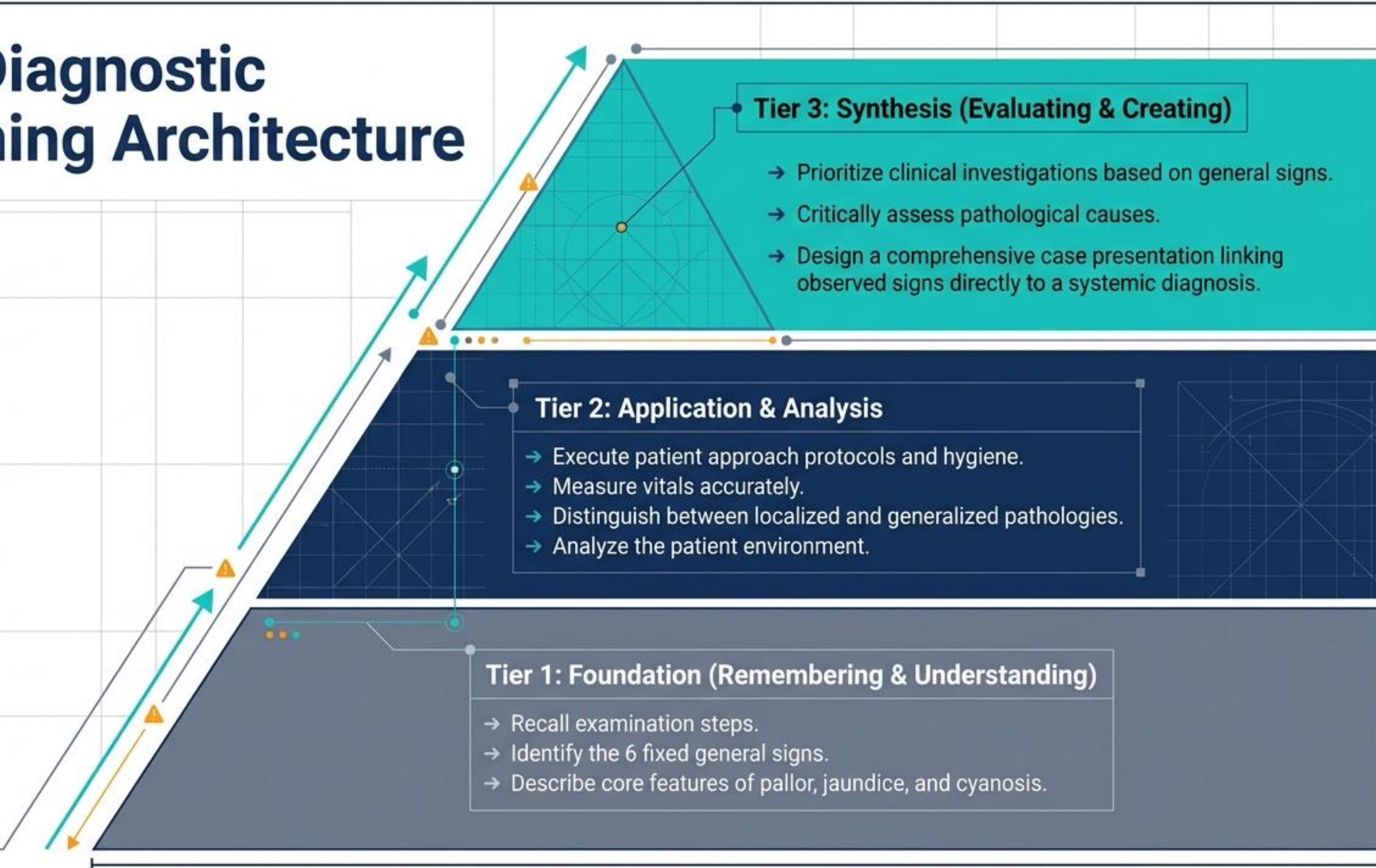
CLINICAL APPROACH & OBSERVATION PROTOCOLS

THE GENERAL PHYSICAL EXAMINATION

A Clinical Field Guide to
Bedside Diagnostics



The Diagnostic Learning Architecture



Tier 3: Synthesis (Evaluating & Creating)

- Prioritize clinical investigations based on general signs.
- Critically assess pathological causes.
- Design a comprehensive case presentation linking observed signs directly to a systemic diagnosis.

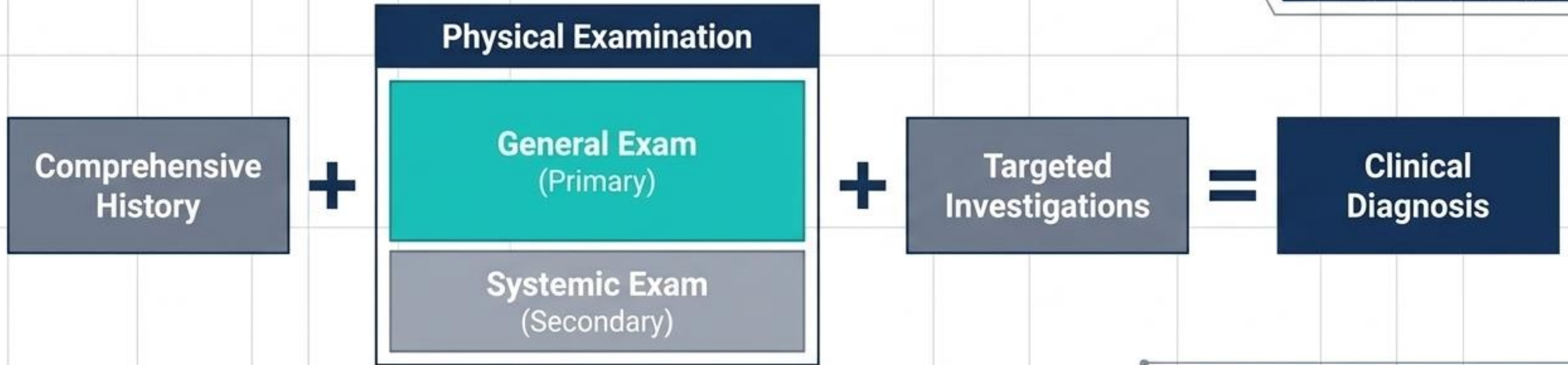
Tier 2: Application & Analysis

- Execute patient approach protocols and hygiene.
- Measure vitals accurately.
- Distinguish between localized and generalized pathologies.
- Analyze the patient environment.

Tier 1: Foundation (Remembering & Understanding)

- Recall examination steps.
- Identify the 6 fixed general signs.
- Describe core features of pallor, jaundice, and cyanosis.

The Diagnostic Paradigm



Crucial Insight: In the hierarchy of the physical examination, the general examination often yields far more immediate diagnostic value than the isolated systemic examination.

Engagement Protocols: The First 60 Seconds



Step 1: Greet & Introduce

Establish rapport with the patient and any relatives present.



Step 2: Explain

Clearly articulate the planned examination process.



Step 3: Sterilize

Perform mandatory hand hygiene (washing, sanitizing, donning gloves).

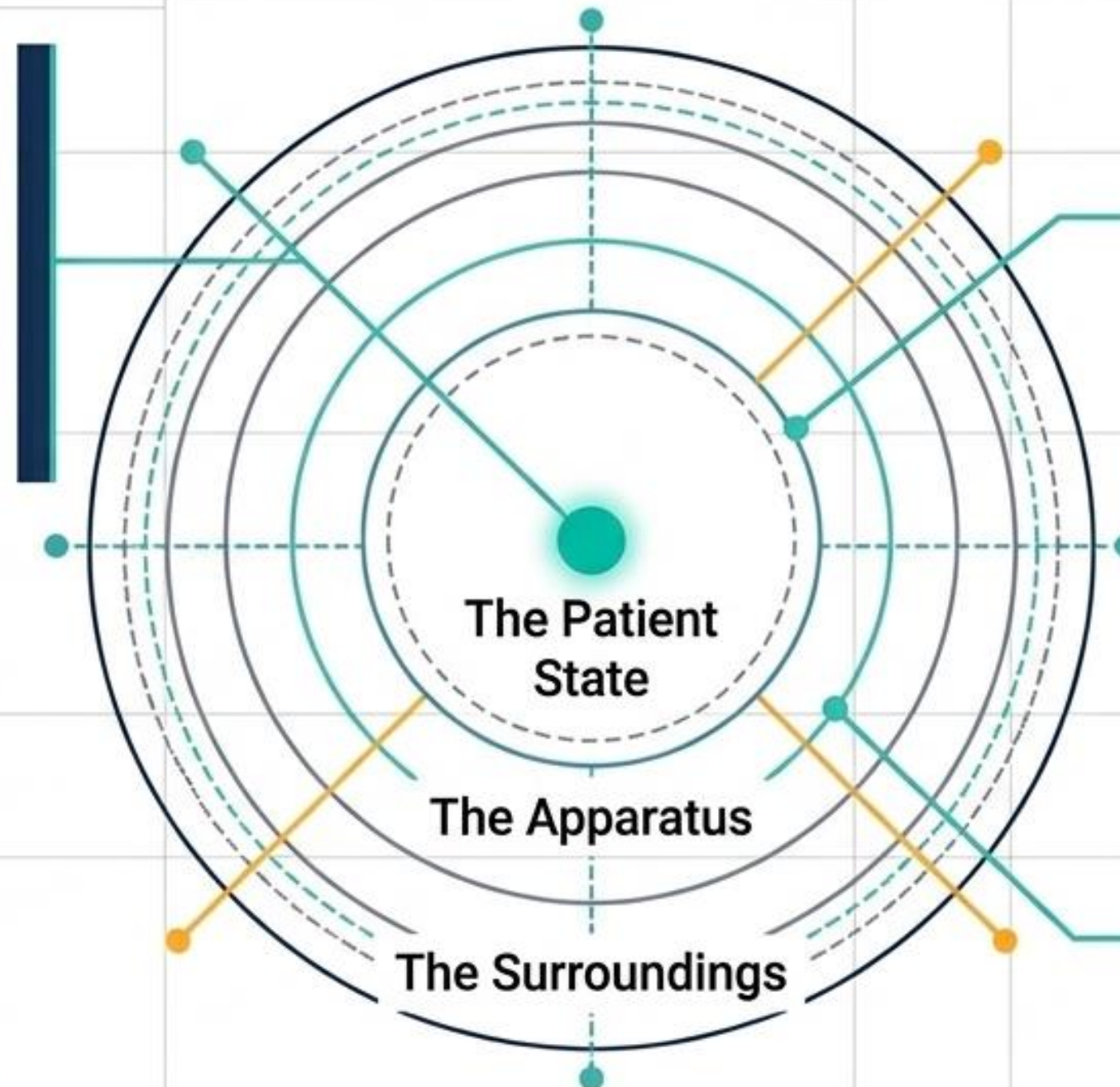


Step 4: Assess Pain

Proactively ask about immediate pain (location, severity) before beginning physical contact.

The Bedside Radar: Environmental & Patient Survey

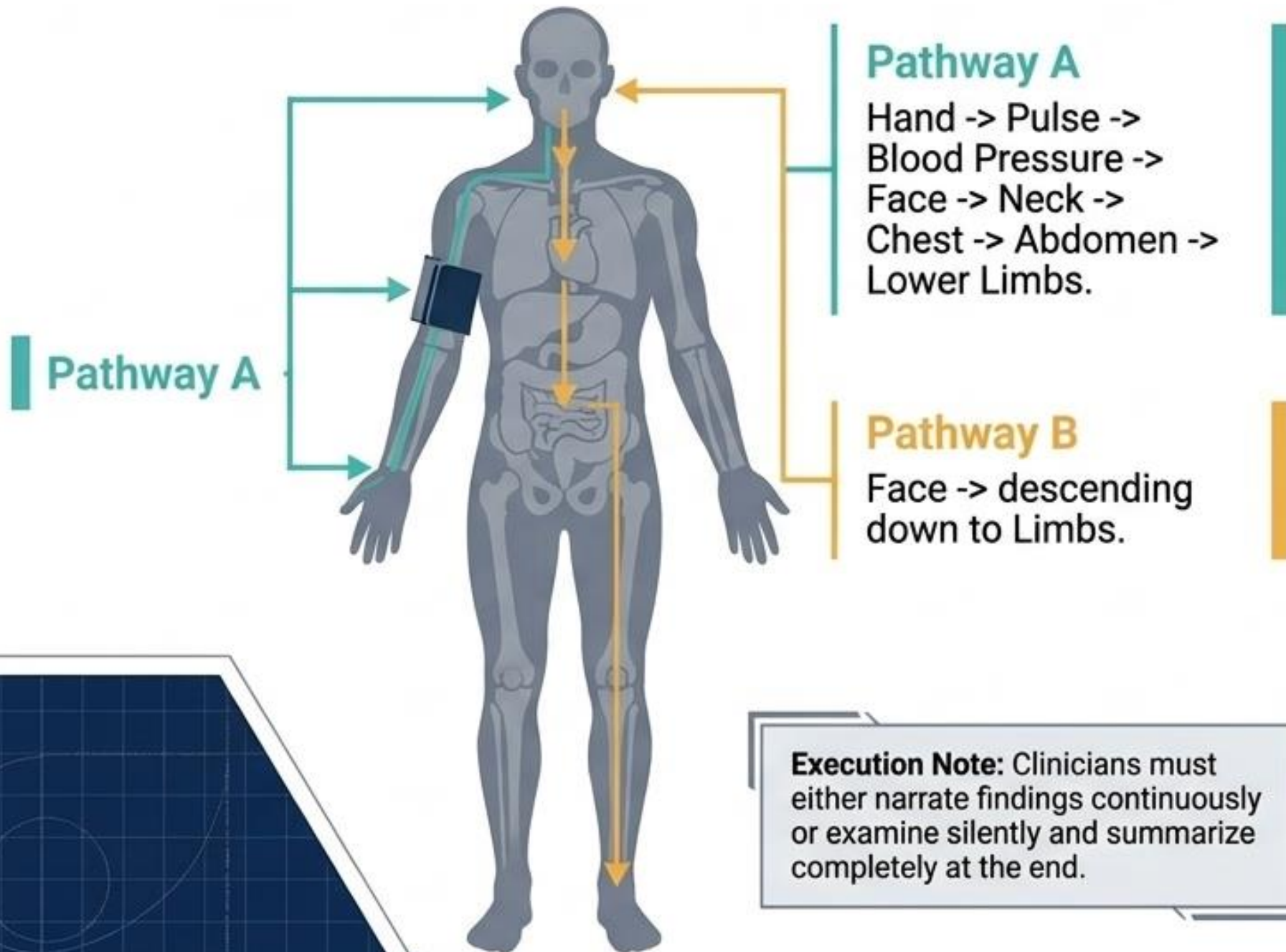
Age, Gender, Facies,
GCS/Level of consciousness,
obvious abnormalities (limb
defects, injuries), eye
contact, audible sounds
(wheeze, cough),
mobility/position in bed.



Use of oxygen, continuous
life support systems, IVs.

Medications present on the
bedside table, active
monitors, nebulizer
machines.

Examination Sequence & Baseline Vitals



Execution Note: Clinicians must either narrate findings continuously or examine silently and summarize completely at the end.



Temperature

37.1 °C



Pulse

72 bpm



Blood Pressure (BP)

120/80 mmHg



Respiratory Rate

16 breaths/min



Oxygen Saturation (SpO2)

98 %

The 6 Fixed General Signs

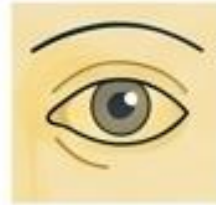
A systematic evaluation of physiological distress indicators

01



Pallor

02



Jaundice

03



Cyanosis

04



Edema

05



Clubbing

06



Lymphadenopathy

Diagnostic Card 01: Pallor

Definition

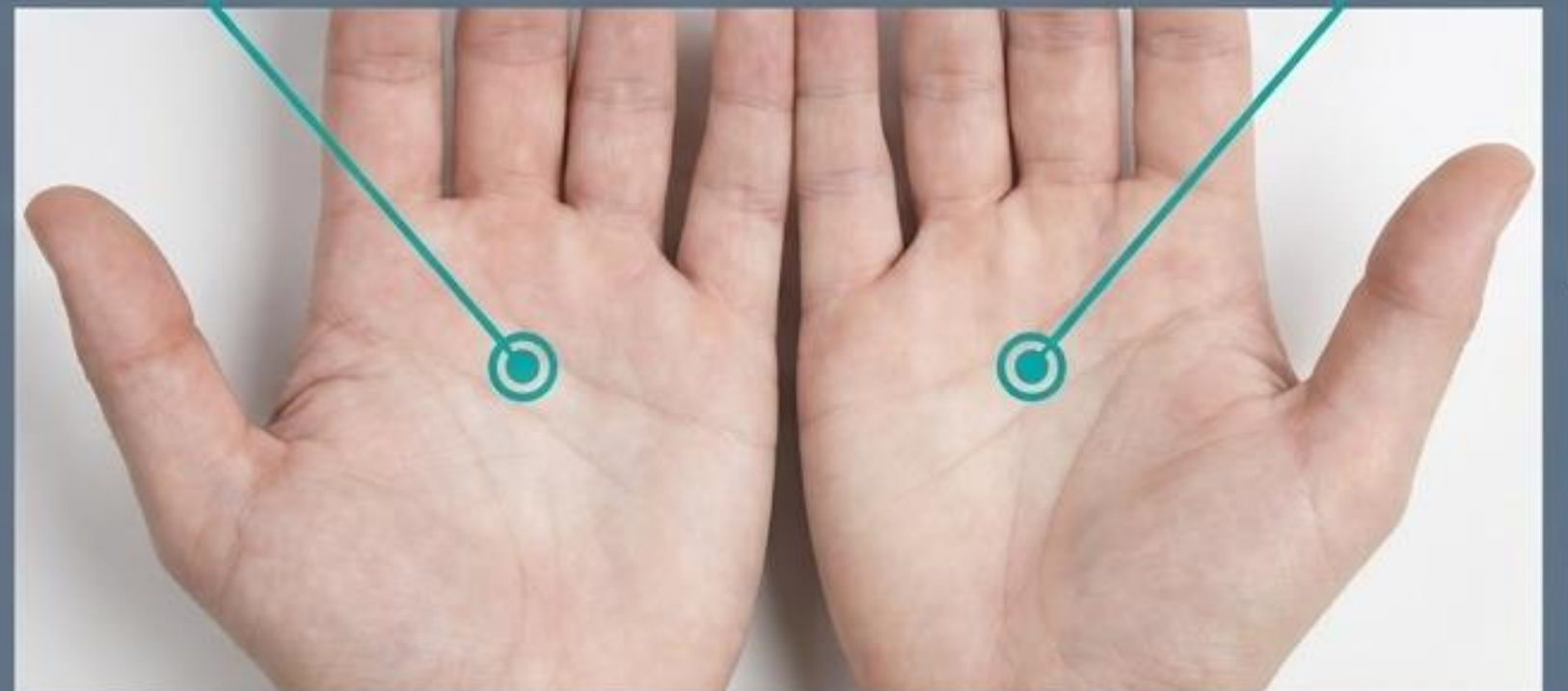
Pale color of the skin, most notably observed in the face.

Underlying Causes

Anemia, acute fear, severe pain.

Key Locations to Check

Palm creases, mucous membranes of the mouth, conjunctiva.



Diagnostic Card 02: Jaundice

Definition

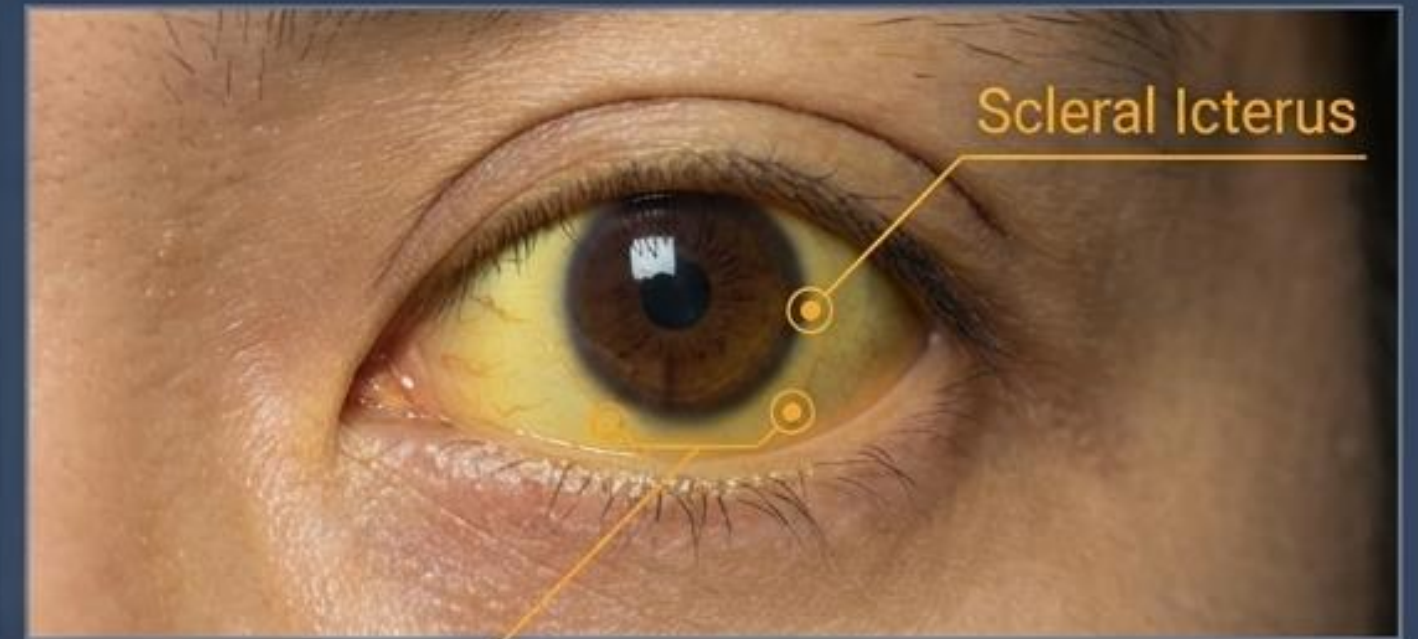
Yellowish discoloration of the skin and mucous membranes.

Mechanism

Occurs when Total Serum Bilirubin (TSB) exceeds the normal physiological threshold for the patient's age.

Key Locations to Check

Sclera, skin, under the tongue.



Diagnostic Card 03: **Cyanosis**

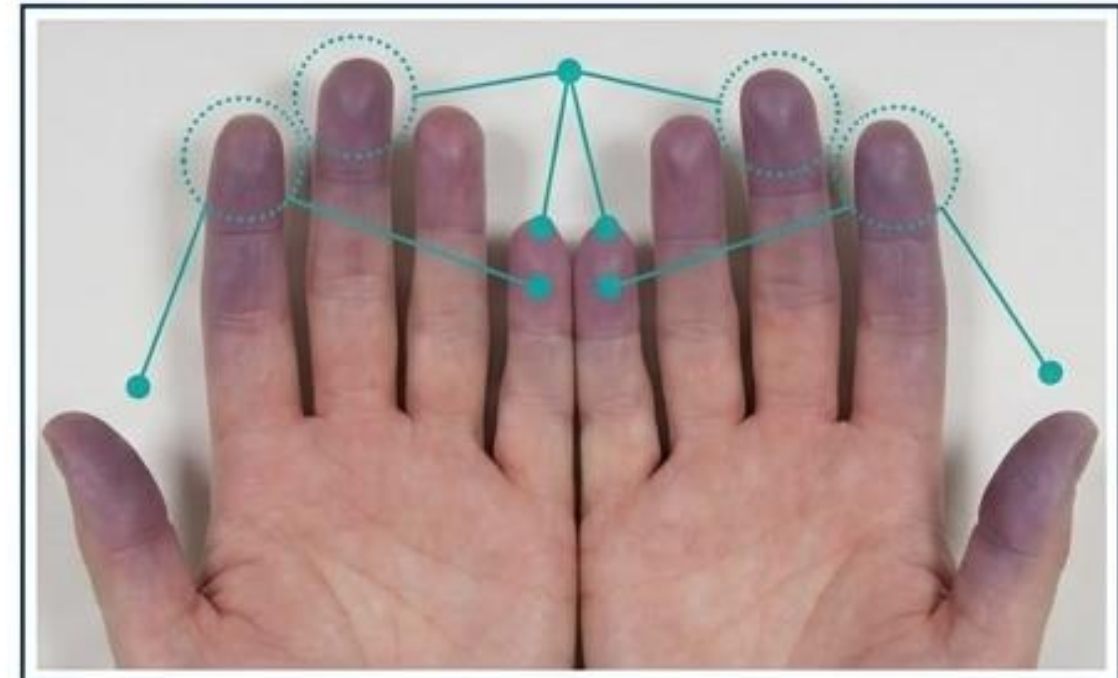
Core Definition: Bluish discoloration in skin & mucous membranes resulting from an increase in desaturated Hemoglobin (Hb) in the blood exceeding 4-5 gm/dl.

Central Cyanosis

- **Classification:** 'Real' Cyanosis.
- **Primary Location:** Seen predominantly in the tongue.

Peripheral Cyanosis (Acrocyanosis)

- **Classification:** Extremity-based.
- **Primary Location:** Seen mainly in the fingers.
- **Primary Trigger:** Frequently induced by cold exposure.

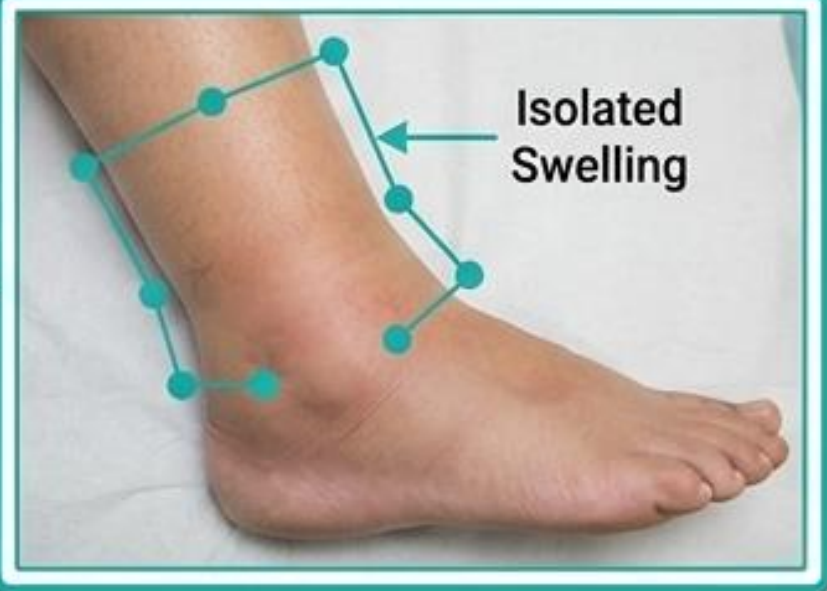


Diagnostic Card 04: Edema (Typology)

Definition: Increase in the amount of fluid in body tissues, primarily within the Extracellular (EC) compartment.

Localized Edema

Mechanism: Driven by local causes (e.g., peripheral lesions).



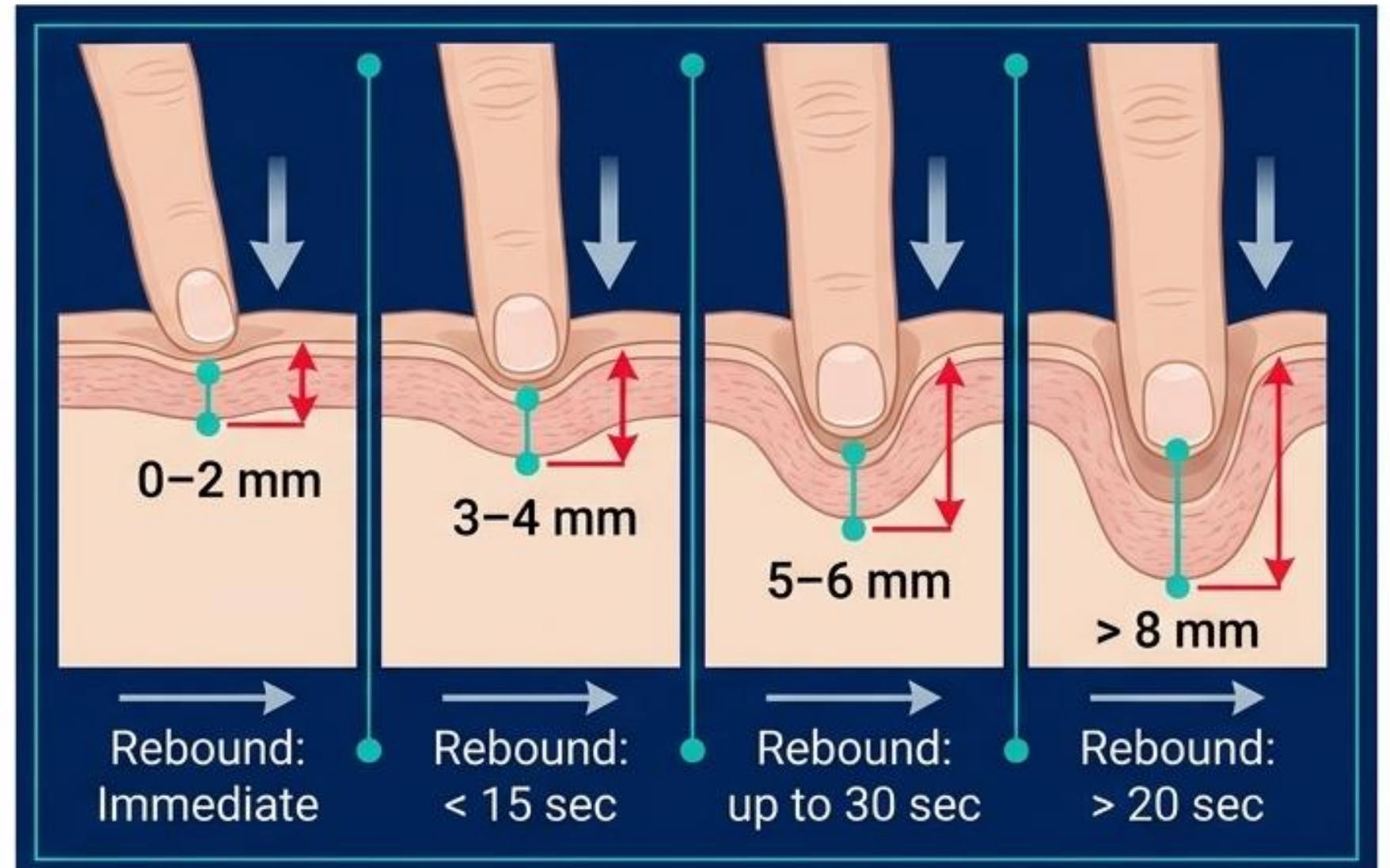
Generalized Edema

Systemic Causes: Hepatic, Cardiac, Renal, Intestinal, Nutritional.



The Physics of Pitting: Edema Grading Scale

Grade 1	0–2 mm indentation; rebounds immediately.
Grade 2	3–4 mm indentation; rebounds in < 15 seconds.
Grade 3	5–6 mm indentation; up to 30 seconds to rebound.
Grade 4	8 mm indentation; > 20 seconds to rebound.



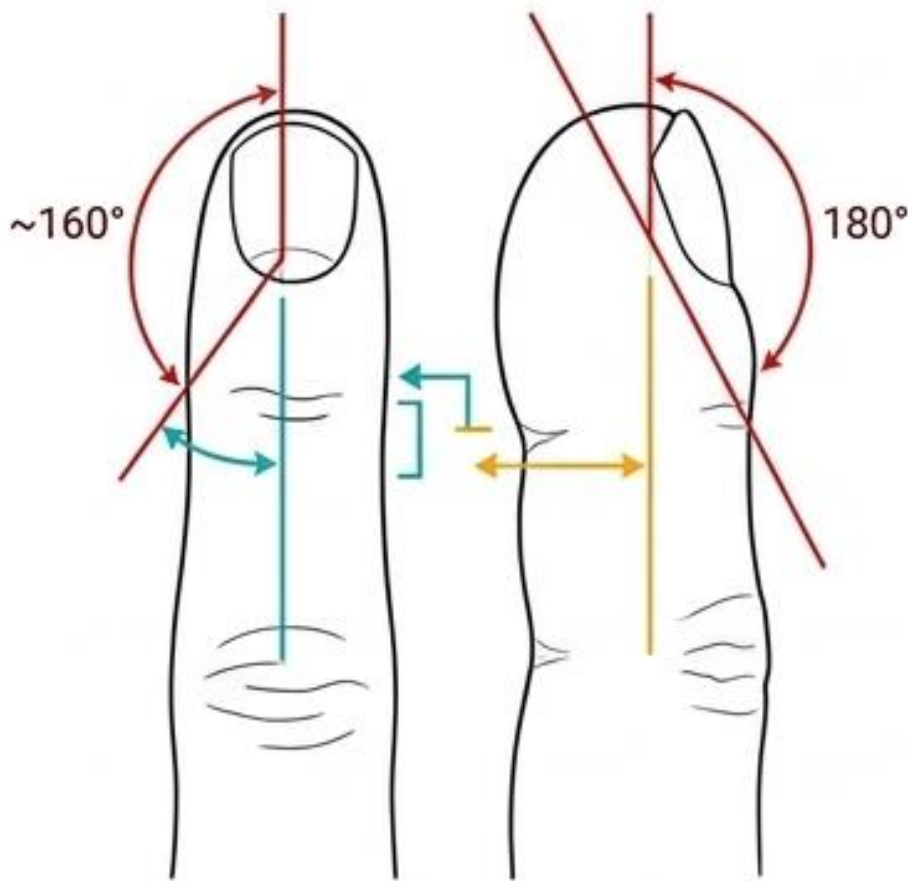
Diagnostic Card 05: Clubbing

Definition: Deformity of the nail beds resulting from Cardiac, Pulmonary, Hepatic, and GIT causes.

Zone 1

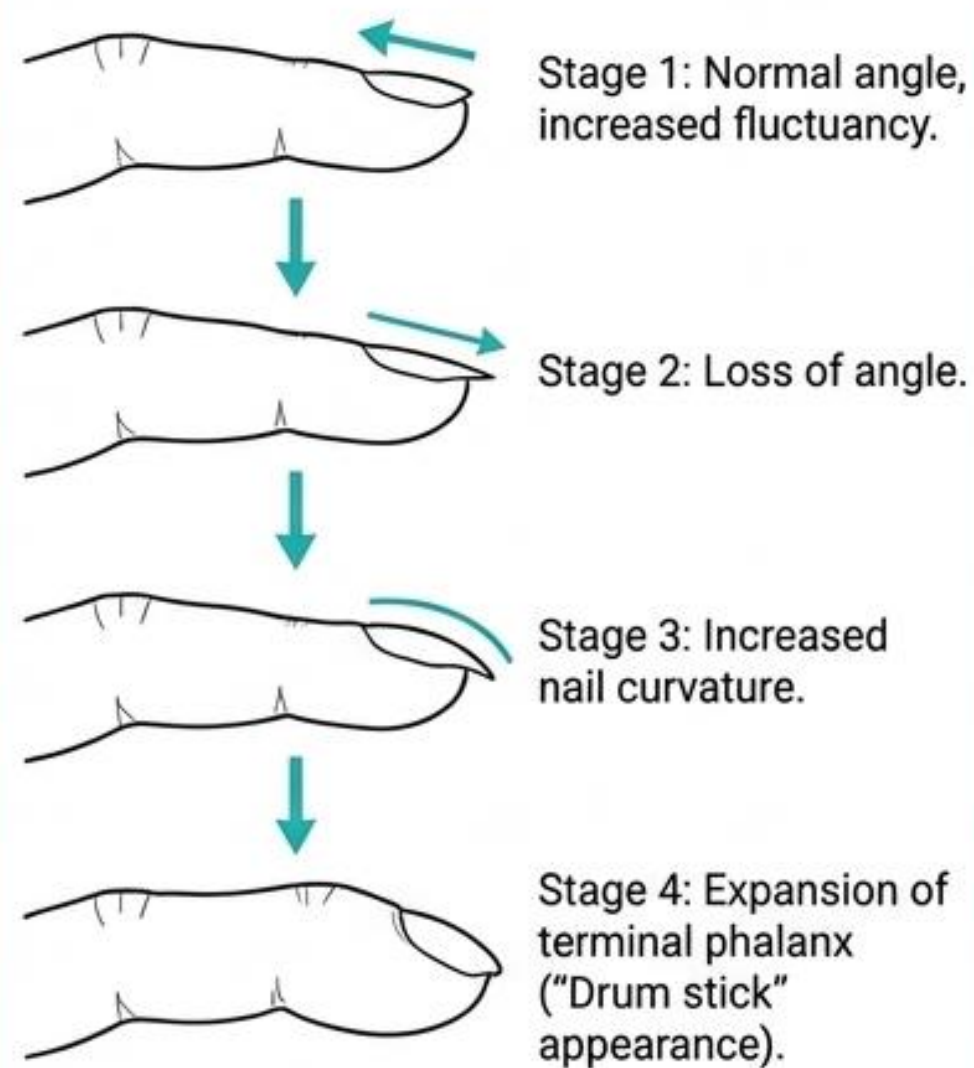
The Geometry

Normal finger angle is $\sim 160^\circ$. Clubbed fingers feature an angle $> 180^\circ$.



Zone 2

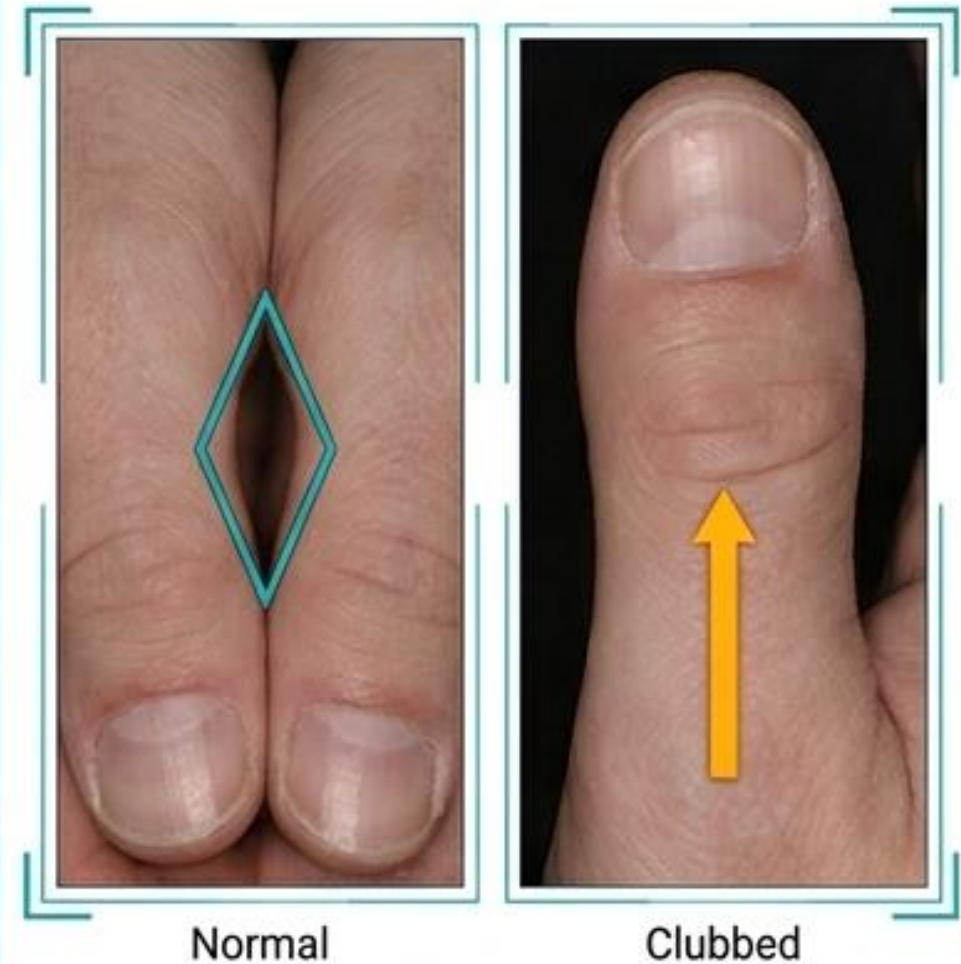
The Progression Stages



Zone 3

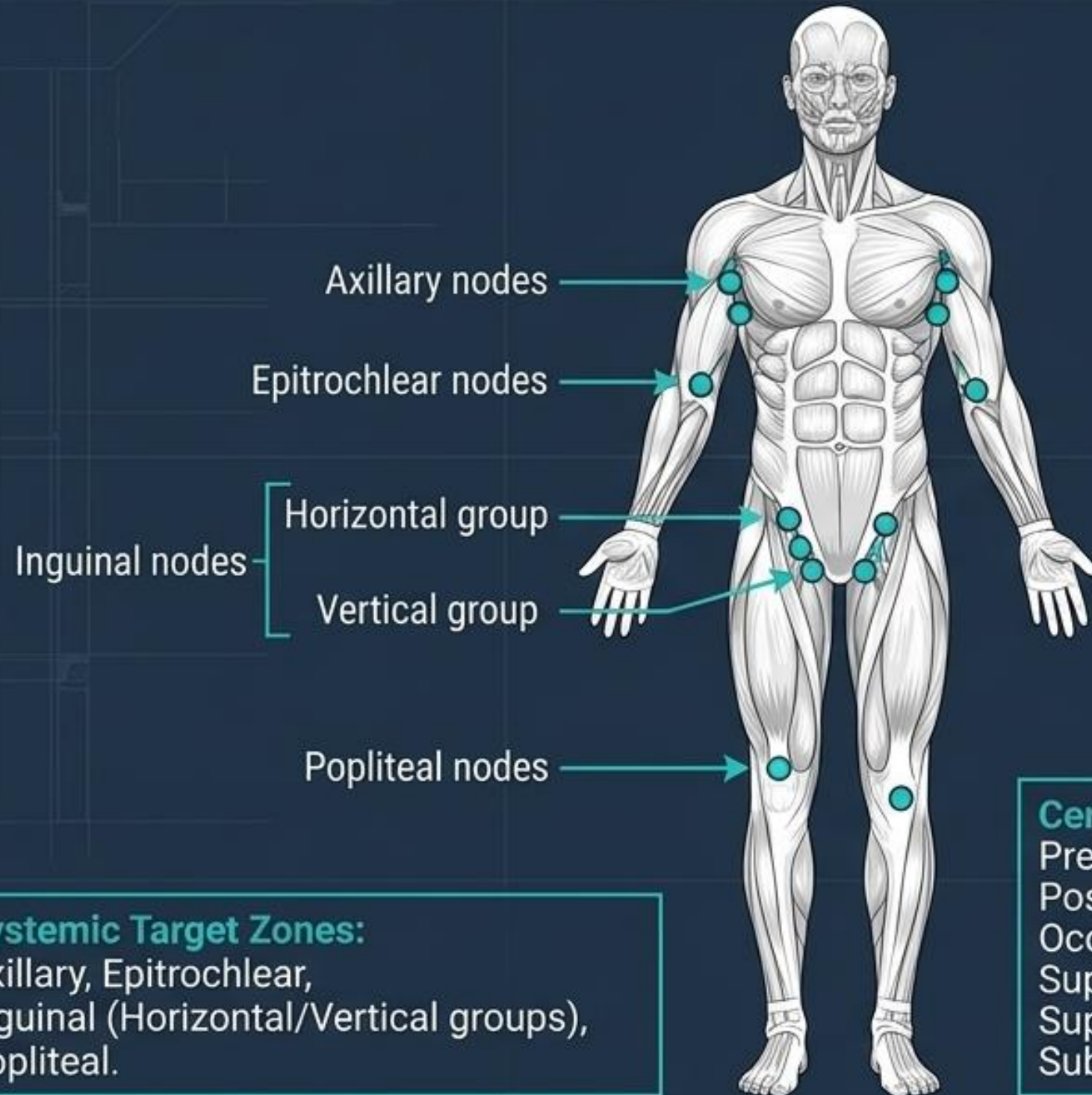
Clinical Test: Schamroth's Window

Clinical Test: Schamroth's Window.



Diagnostic Card 06: Lymphadenopathy

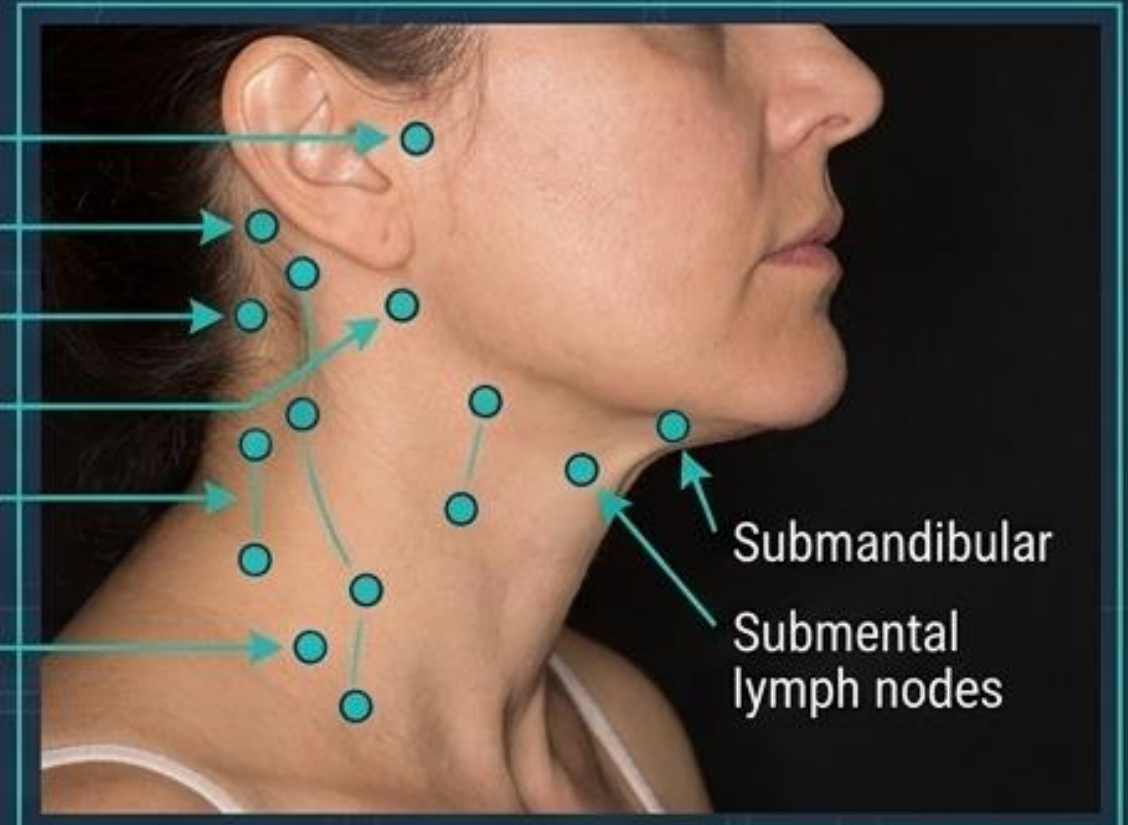
Definition: Examining systemic and localized lymph node groups to identify pathological swelling.



Systemic Target Zones:

Axillary, Epitrochlear, Inguinal (Horizontal/Vertical groups), Popliteal.

Preauricular
Posterior auricular
Occipital
Tonsillar
Superior/Posterior cervical
Supraclavicular



Cervical Target Zones:

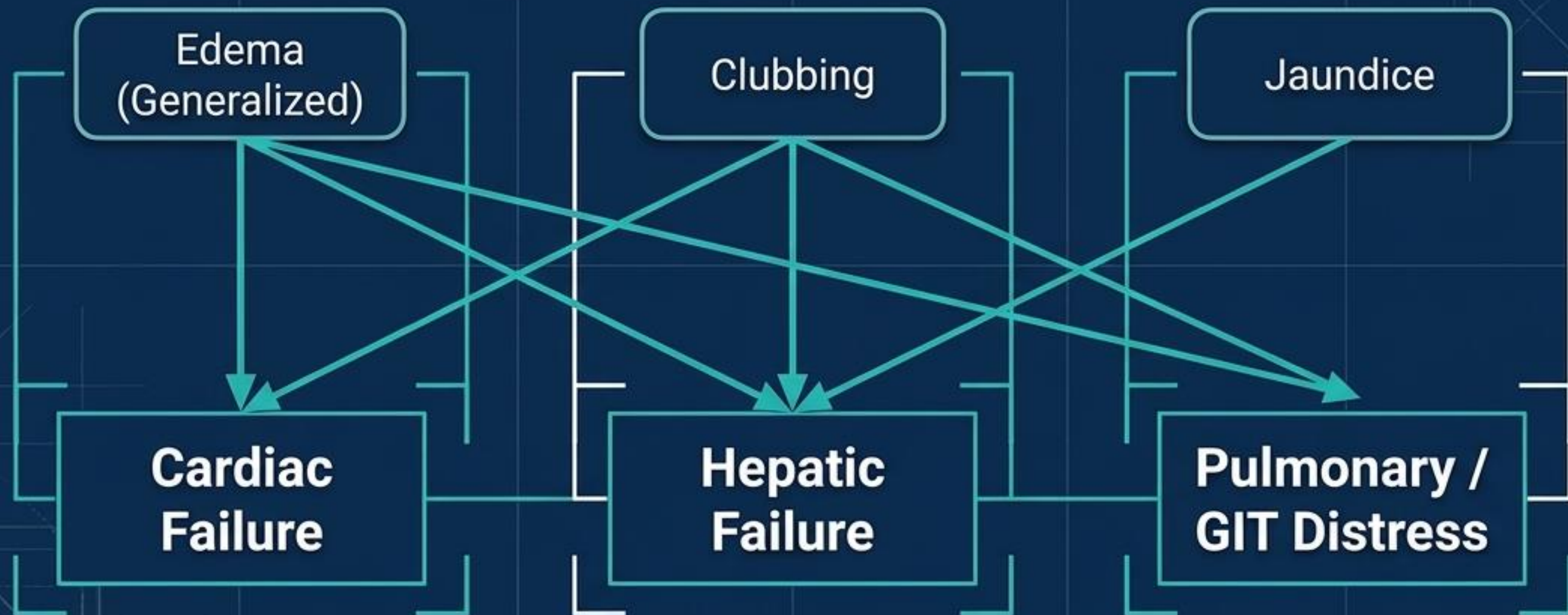
Preauricular, Posterior auricular, Occipital, Tonsillar, Superior/Posterior cervical, Supraclavicular, Submandibular, Submental.

Swollen Lymph Node Mass



Synthesis: From General Sign to Systemic Verdict

Insight: General physical signs rarely exist in isolation. Their intersections form precise diagnostic blueprints.



Concluding Takeaway: The rigorous, standardized application of the General Physical Examination remains the clinician's most powerful tool for early systemic diagnosis.